



Clozapine Prescribing, Administering, Monitoring and Capillary Sampling SOP

Scope

Site	Department, division or operational area	Applicable to
Royal Perth Bentley Group (RPBG)	Mental Health Division (MHD), Inpatient wards and Community Mental Health Services (CMHS)	Medical, Nursing, Pharmacy, Clerical

Purpose

This standard operational procedure (SOP) aims to provide clinicians with guidelines to ensure the safe prescription, monitoring and administration of Clozapine to consumers within the in-patient and Community Mental Health Settings, and the use of the HemoCue® machine in accordance with the manufacturer operational manual.

Position statement

Clinicians must practice within the Mental Health Act 2014 (MHA) and the [Charter of Mental Health Care Principles \(MHA 2014\)](#)

For Respiratory Virus Infection (RVI) precautions and guidance, refer to the [CMHS Provision Responding to Respiratory Virus Illness SOP](#) and [RPBG RVI and Infection Prevention and Management Clinical Guideline](#).

Links within document

- [Appendix I: Bidi Wungen Kaat \(Transitional Care Unit\) Clozapine Monitoring Guidelines](#)

Background

Clozapine is an effective antipsychotic medication indicated for use in the management of treatment resistant schizophrenia in cases where consumers are non-responsive or suffer intolerable side effects to at least two antipsychotics from at least two different groups (i.e. typical and atypical antipsychotics) other than clozapine.

Due to the risk of neutropenia and agranulocytosis, all consumers taking clozapine are enrolled in a registry and monitored regularly for adverse effects. Prescription is initiated while the consumer is an inpatient and then care transitioned to a CMHS, where the consumer will be managed by the relevant Mental Health Clozapine Clinic.

There are 3 brands of Clozapine available in Australia: Clopine®, Clozaril® and Clozitor®. Currently the preferred brand of Clozapine used by the Department of Health Western Australia is Clopine®. However, the medication tender for the state is reviewed periodically, which may affect future prescribing requirements.

Background cont'd

Consumers attending the Clozapine Clinic utilise various pathology centres to have their routine clozapine bloods taken. The HemoCue® machine allows stable consumers who have usual blood test results within the green therapeutic range to attend the community clinic for capillary blood sampling, however, this is to be used only when necessary (in the event of missed blood tests or urgent testing requirements), as venous samples are the preferred method of haematological monitoring.

Definitions

Capillary Sampling	Refers to sampling blood from a puncture on the finger. It is less invasive than venous puncture, requires smaller amounts of blood volume and can be performed quickly and easily.
Clozapine clinics	Specialise in the monitoring of consumers on clozapine in the community Geographically are usually site based or in exceptional cases, mobile and accessed by the consumer as per their agreed individual management plan Provide a comprehensive mental health service for adults who are prescribed clozapine.
Clopine Centre	A Clopine Centre is defined as a hospital, clinic or other health facility that is involved with the use of Clopine®, in conjunction with ClopineCENTRAL. Consumers are listed as 'belonging' to a specific Centre through their registration at the Centre.
Clopine Centre Coordinator	An identified staff member (commonly nursing or pharmacist) who has administrative access to the ClopineCentral Centre who oversees and facilitates successful adherence to protocols at the Centre including ensuring that required blood tests are carried out for each consumer, the results checked promptly, and appropriate action is taken whenever necessary.
Clozapine nurse	An identified registered nurse who coordinates the clozapine clinic and is responsible for undertaking clinical review, physical observations and HemoCue capillary sampling of the consumers attending the clozapine clinic. This clinician can also act as the Clopine Centre Coordinator (as above) or be a separate role.
HemoCue® white blood cell differential (WBC DIFF) system	Used for the quantitative determination of the total number of white blood cells in capillary or venous whole blood. The single use microcuvette serves as a sample container and reaction chamber. The blood sample is drawn into the microcuvette by capillary action and after the microcuvette is placed in the analyser a result is obtained within five minutes.



Contraindications and precautions

- A. Contraindications: include severe hepatic, renal or cardiovascular disorders including active hepatic disease associated with nausea, anorexia or jaundice, progressive hepatic disease and hepatic failure.
- B. Special Precautions for the use of Clozapine include:
 - **Pregnancy:** Clozapine is only recommended for use during pregnancy when the benefit of treatment outweighs the risk that inadequately controlled psychiatric illness might pose to both mother and embryo/foetus
 - **Breastfeeding:** Since clozapine is excreted in breast milk, mothers receiving clozapine should be informed of the possible risks whilst considering the benefits of breastfeeding
 - **Older People:** Use in older consumers requires a lower dose at initiation of treatment, and the dose titrated up more slowly as older people are more susceptible to side effects
 - **Seizures / Renal / Liver Impairment:** Clozapine is typically initiated at a dose of 12.5mg. In high-risk consumers with a history of seizures, or suffering from cardiovascular, renal or hepatic disorders, dose increases should be titrated more slowly and in smaller increments (contact ClopineCentral for dosing advice if required).
 - **Smoking and caffeine:** A change in smoking and caffeine intake can affect clozapine metabolism, which may lead to changes in clozapine levels and risk of toxicity:
 - Smoking:
 - Tobacco smoke (containing polycyclic aromatic hydrocarbons) increases clozapine metabolism, resulting in decreased clozapine levels
 - Cessation of smoking increases clozapine levels which may result in toxicity
 - Nicotine replacement therapy (NRT) does not affect clozapine levels
 - Caffeine:
 - Caffeine increases clozapine levels which may result in toxicity
 - Increased consumption of caffeine containing energy drinks e.g. cola may result in elevated clozapine levels
 - Share this information with the consumer/relevant others and document baseline smoking and caffeine intake habits which must be updated at each clinical review. Any change in smoking and caffeine consumption must be reported to the prescribing clinician
 - For further information on important drug interactions with clozapine, recommencing therapy after an interruption and discontinuing clozapine therapy refer to ward pharmacist, RPBG medicines information pharmacist, or ClopineCentral
 - For haematology advice for consumers, contact ClopineCentral on-call Haematologist T: (03) 9387 1000. 24 hours/day, 7 days/week. If call is unanswered, leave a message with full name, designation, contact number and enquiry details
 - **All other queries/advice contact ClopineCentral™**
 - Telephone: 1800 656 403 / Fax: 1800 657 454 / E: Clopinecentral@pfizer.com
- C. Concomitant use of depot antipsychotics is discouraged

Common side effects

- Refer to the below table that outlines the more common side effects related to clozapine. For a list of pharmacological options and actions, the treating team should contact the clinical pharmacist and refer to the [Australian Medicines Handbook](#)¹

Table 1: Side-effects associated with clozapine therapy¹

Common (greater than 1%)	Infrequent (0.1–1%)	Rare (Less than 0.1%)
- drowsiness (occurs in 40%) - hypersalivation (can cause aspiration pneumonia) - constipation (may result in obstruction, paralytic ileus and death) - seizures - headache - tachycardia - hyperpyrexia (5%) - hepatitis - neutropenia - vomiting - urinary incontinence - nocturnal enuresis	- myocarditis (usually in the first month of initial treatment but rarely may occur when starting after a break in treatment) - agranulocytosis - eosinophilia - priapism - extrapyramidal side effects (EPSE)	- cardiomyopathy - hypertension - myoclonic jerks - interstitial nephritis - respiratory arrest - fulminant hepatic necrosis - polyserositis

- Provide consumers with [Consumer Medicine Information \(tga.gov.au\)](#) as per requirements in the Medication Review Policy. Patient information in alternative formats and languages are available via [Choice and Medication website](#).

Management of clozapine related side-effects

- The metabolic effects of clozapine (and other antipsychotic medications) along with increased weight and waist circumference are well known. Clinicians are to provide all consumers on clozapine with education on the benefit of maintaining a healthy diet and regular exercise. Consider referral to specialist services for weight management and diet advice where appropriate.
- Constipation and gastrointestinal hypomotility are recognised as common adverse side effects of clozapine. Clinicians are to ensure direct questioning about bowel movements are included in their consumer assessments, and education is provided on maintaining healthy bowel habits. The RPBG produced “My Bowel Diary” is a resource that is recommended for consumer use which helps consumers track and manage their bowel health and can be ordered via the Print Media Group. The prescription of aperients should be considered early in any patient who experiences bowel changes on clozapine.
- Regular communication (6 monthly minimum) with General Practitioners is recommended as per the *Clozapine Monitoring Form Part B*, particularly where a consumer requires prescription of medication to manage metabolic syndrome (e.g. Metformin) and to ensure oversight of metabolic blood testing results.

Blood testing

- A full blood picture (FBP) including WCC and ANC must be performed for all consumers on Clozapine and must occur weekly for consumers who are initiating Clozapine and within their first 18 weeks of treatment, or has had a therapy interruption of more than four weeks or monthly for consumers who have completed their first 18 weeks of therapy and are considered in maintenance. Blood tests results >48 hours prior to the community appointment are not valid and cannot be utilised in order to issue a prescription for Clozapine.
- Clozapine blood results monitoring system is divided into 3 defined ranges for WCC and ANC as in the table below:

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State	WBC	Neutrophil	Outcome
Green	≥ 3.5	≥ 2.0	Continue with regular blood
Amber	≥ 3.0 and < 3.5	≥ 1.5 and < 2.0	Twice weekly blood tests required
Red	< 3.0	< 1.5	Cease therapy immediately

- If the consumer's blood results return in the green range, they can be prescribed the full amount of Clozapine until their next blood test.
- If the consumer's blood results return in the amber range, Clozapine therapy can continue at the usual dosage but blood testing and clinical review must be increased to TWICE a week until their blood results return to the green range. They must be supplied with only 2-3 days quantity of medication during this amber period until their next blood test.
- If the consumer's blood results return in the red range, clozapine therapy must be discontinued immediately and reported to the relevant patient monitoring system; Clopine Central™. Provide consumer education on the importance of ceasing clozapine and monitoring for adverse effects. If consumer using dose administration aid, liaise with community pharmacy to ensure clozapine supply is removed.
- Clopine Central™ offers haematology advice for any consumer on Clozapine therapy. The Clopine® haematologist can assist with any haematology queries as well as altering WCC and ANC thresholds for specific consumers (e.g., consumers on active chemotherapy or benign ethnic neutropenia where thresholds may need to be lowered).
- Additional blood tests including Clozapine levels may be requested based on additional monitoring requirements or at the discretion of the treating doctor.

Inpatient Commencement of Clozapine (or recommencing after interruption of more than 28 days)

- Clozapine must be commenced in an inpatient facility due to the requirement for close physical monitoring and the dose titration that is required during the first two weeks of treatment
- For specific Bidi Wungen Kaat (Transitional Care Unit) monitoring guidelines please see [Appendix I](#). Clozapine initiation should take place early in the week (Monday or Tuesday only). Dose titration may be slower than the recommended schedule as shown on the *WA Clozapine Initiation and Titration Chart* to support the **TCU Model of Care and staffing profile**. It is suggested that dose changes do not occur on weekends, public holidays and the day preceding weekends and public holidays as there is limited medical support. Dose titration should be individualised – please refer to the treating psychiatrist.
- Ensure that all consumers complete the *Consent to Clozapine Treatment Form* prior to commencement (unless detained under the MHA 2014). All efforts should be made to ensure consumers are re-offered consent forms once considered able to give informed consent.
- Clozapine can only be initiated by a Consultant Psychiatrist. Medical Officers working in inpatient units can initiate or continue prescribing clozapine on the advice and supervision of a Consultant Psychiatrist, if it is medically safe to do so.
- The Medical Officer (MO) only to:
 - Prescribe initiation of clozapine on the *WA Adult Clozapine Initiation and Titration Chart* (refer to the chart for all dosing and monitoring guidelines)
 - Liaise with inpatient pharmacist to register the patient with Clopine Central prior to initiation
- All consumers will have the following baseline investigations done prior to their clozapine commencement/re-commencement (within 7 days of commencement):

○ Full Blood Count (FBC) ○ Urea and Electrolytes ○ Blood group (BG) ○ Fasting glucose (BGL) or HbA1c ○ Fasting lipids ○ Liver Function Tests (LFT) ○ C-reactive protein (CRP) ○ Troponin (TROP)	○ Echocardiogram (Echo) and 12 lead Electrocardiogram (ECG) ○ Physical examination ○ Weight + Height (calculate BMI) ○ Waist circumference ○ Vital signs ○ Record current smoking status, caffeine intake, lifestyle and diet
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- Investigation potential problems with prescribed, purchased, or herbal medication, illicit drugs and alcohol
- Treating team to ensure that on commencement of clozapine a Clinical Alert on webPAS and PSOLIS/WebPSOLIS (medication alert) are added to these respective systems. This ensures oversight of the consumer's clozapine regime in the event of presentation to another health service.

Inpatient monitoring requirements

Clinical staff (nurse or MO) are to:

- Record all consumer vital signs during initiation as per the *WA Adult Clozapine Initiation and Titration Chart* and also concurrently on the local *Adult Observation and Response Chart (AORC)*
- Ensure all pathology tests and investigations are completed at their required intervals as per the *WA Adult Clozapine Initiation and Titration Chart*
- Commence a *Clozapine Monitoring Form Part A* on initiation. Form to be completed once per week whilst consumer is on weekly bloods (then monthly thereafter). Refer to form for side effect assessment guides and all other monitoring requirements.
- Ensure ongoing vital signs (post initiation) are completed as per local guiding policy or procedures and are recorded on the local *Adult Observation and Record Chart (AORC)*
- Continue to complete the *Clozapine Monitoring Form Part A* at weekly or monthly intervals (depending on weekly or monthly blood test requirements) including assessment of side effects
- Commence *Clozapine Monitoring Form Part B* at 4 weeks post initiation and ensure all monitoring and investigations are completed as per the form.

Escalation guidelines for suspected clozapine related side effects (inpatient)

- Escalation of any noted physical health deterioration must be (at a minimum) as per the local AORC escalation guidelines and Code Blue policies.
- Increase frequency of observations based on AORC chart guidelines or as determined by the treating medical team. Any refusal of vital sign observations in a consumer suspected to have Clozapine related side effects should be escalated immediately to the treating medical team for review and investigation. Treating team are to consider:
 - Refer to RPBG Cardiology for advice and treatment options
 - Clopine® Haematology service can also be contacted for clinical advice
 - Consider if the patient is appropriate for leave, or to stay on ward
 - Withhold Clozapine until full review and investigations have been completed
 - Ensure that clozapine is ceased if there is confirmed myocarditis or agranulocytosis (related to clozapine).

Discharge from inpatient admission to CMHS

Clinical staff (nurse or MO) are to:

- Ensure the RPBG MHD ward/unit specific discharge process is followed and complete all documentation in accordance with existing local policies
- Contact CMHS taking over care of consumer and confirm Care Coordinator. Document outcome in Transfer Discharge Summary
- Confirm that the inpatient pharmacist has transferred to consumer details to the correct community ClopineCentral centre
- Send clozapine specific information to CMHS (unless digital versions are available for the community team [i.e. Digital Medical Record site])
 - Copy of *WA Adult Clozapine Initiation And Titration Chart* (if consumer's clozapine initiated during admission)
 - Copy of *Inpatient Clozapine Monitoring Forms* (A, B and C) if commenced
 - Latest ECHO, ECG and most recent blood results
 - Copy of the *Consent to Clozapine Treatment Form* (for consumers who've commenced clozapine if completed/available)
- Confirm and communicate to the community team when next blood test is due (date) and how many days of clozapine medication supply the consumer is discharged with
- Communicate with CMHS to ensure appointment for review is booked appropriately
- Provide consumer with blood test request form. Hemocue testing should not be utilised whilst consumer is on weekly monitoring.

Clozapine monitoring in the community

- Clinic based assessments are to be completed on a weekly or monthly basis, aligning with the consumers blood test due dates to ensure continuity of prescription and supply of Clozapine. The Clozapine nurse will facilitate the clinic based assessment, where the consumers physical health monitoring will occur as per the *Clozapine Monitoring Form Part A and B (or C)*. Consumers will also have their mental state assessed and monitored by either the Clozapine nurse, or their treating doctor, along with their blood tests results reviewed and their prescription and blood count form supplied
- Consumers who are assessed as mentally stable, on monthly blood testing, and without Amber or Red range blood tests in the last 3 months are considered suitable for Clozapine nurse only appointments but must still be reviewed by their treating doctor at least every 3 months. All other consumers must be seen monthly by their treating doctor
- Community based assessments that are conducting in care facilities (not in the community clinics) must follow the same principles as clinic based assessments, including completion of the *Clozapine Monitoring Forms*, along with physical health monitoring and medical review.

Escalation guidelines for suspected clozapine related side effects (community)

- A community based mental health consumer who is experiencing suspected clozapine related side-effects or acute deterioration must (at a minimum) have their care escalated as per local deterioration guidelines and medical emergency or Code Blue policies.
- Increase frequency of clinical review as determined by the treating doctor. Any refusal of vital sign observations in a consumer suspected to have Clozapine related side effects should be escalated immediately to the treating Consultant or Medical Lead for review and investigation. Treating doctor is to consider:
 - Refer to RPBG Cardiology for advice and treatment options.
 - Clopine® Haematology service can also be contacted for clinical advice
 - Consider if the patient is appropriate for community care, or requires transfer to a tertiary service or inpatient admission
 - Withhold Clozapine until full review and investigations have been completed
 - Ensure that clozapine is ceased if there is confirmed myocarditis or agranulocytosis (related to clozapine)

Discontinuing Clozapine

- Discontinuing clozapine abruptly carries the risk of cholinergic rebound syndrome resulting in profuse sweating, nausea, vomiting, diarrhoea and headache. The patient's mental state should also be monitored carefully due to risk of psychosis. A gradual reduction of clozapine over a 1-2 week period can reduce this risk. Once Clozapine has discontinued, the Clozapine monitoring service needs to be notified. This can be done through the Clopine Central™.
- Post monitoring blood tests:
 - **Weekly Monitoring:** Testing must occur at time of discontinuation and continue weekly for at least 4 weeks
 - **Monthly Monitoring:** Testing must occur close to time of discontinuation and then four weeks later
 - **Twice-weekly monitoring due to amber result:** Patients who are being monitored twice weekly due to blood results in 'amber' range and who have discontinued clozapine should continue twice weekly blood tests until blood results return to 'green' range. Once in 'green' range, continue post monitoring blood tests to reflect prior monitoring schedule i.e., weekly monitoring or monthly monitoring. See above.

HemoCue® Capillary sampling

Specimen

Whole blood sample collected by fingerstick (venepuncture samples will not be tested in the clinic).

Inclusion criteria

Consumers:

- Who are prescribed Clozapine medication
- Whose most recent blood test was within the Green range
- Who have completed the initial 18-week Clozapine titration phase.

Exclusion criteria

Consumers:

- Who refuse the HemoCue® test
- Whose most recent blood test was within either Red or Amber range
- On weekly monitoring (initial 18 weeks or re-titrating).

Procedure details

The Clozapine Coordinator/Clinic Nurse will:

1. Ensure awareness of consumer's risk history and management plan
2. Obtain and document the consumer's once-off consent for capillary sample testing
3. Enter operator and consumer ID into the HemoCue® machine, ensuring consumer details are correct before proceeding
4. Ensure the printer is connected before performing analysis
5. Perform hand hygiene and prepare the consumer. Don non-sterile gloves as per standard precautions prior to taking capillary sample
6. If the consumer declines the procedure, determine if the consumer understands the risks associated with not providing a blood sample and potential interruption of therapy. In liaison with the consumer's Care Coordinator make alternative arrangements to obtain a blood sample by providing a pathology request form and instructing the consumer to attend pathology centre.

Taking a capillary sample:

The Clozapine Coordinator/Clinic Nurse will:

1. Prepare equipment and check expiry date of microcuvette
2. Confirm consumer's identification
3. Ensure the consumer is seated comfortably and that the consumer's hands are warm and relaxed prior to commencing the procedure
4. Perform hand hygiene prior to donning non-sterile gloves
5. Use the consumer's middle or ring finger for sampling. If possible, rings should be removed from sampling finger
6. Request consumer to
 - Use their thumb, lightly press their finger from the top of the joint to the tip. This stimulates the blood flow towards the sampling point
 - Move their thumb to the tip of the finger and squeeze.

HemoCue® Capillary sampling - Taking a capillary sample1 cond't

7. Clean sample finger with alcohol swab, ensuring finger is dry before lancing
8. Lance finger whilst applying pressure of the consumer's finger and prick the side of finger closest to thumb
9. Apply gentle pressure against last finger joint to create large blood droplet.
10. Release pressure and wipe away first 2 - 3 drops of blood. This stimulates the blood flow and removes any excess interstitial fluid that would be present in the initial drop of blood
11. If necessary, apply light pressure again until another drop appears. It is vital to achieve a free flow of blood. Avoid "milking" from the base of the finger as this may dilute the specimen with plasma and alter the result
12. Place the cuvette tip into the middle of the drop of blood and fill in one continuous action (refer to [Figure 1](#))
 - a. The sample will be drawn up automatically into the cuvette via capillary action
 - b. Ensure that the cuvette fills entirely, and there are no bubbles evident; if present, dispose of the cuvette and take a new sample from a fresh drop of blood
 - c. DO NOT attempt to top up cuvette (double dip) or refill
13. Wipe off the excess blood on the outside of the cuvette tip with a lint free wipe (e.g. cotton pad) by wiping cuvette towards the tip. Take care that no blood is drawn out of the cuvette in this procedure. Do not wipe the open side of cuvette

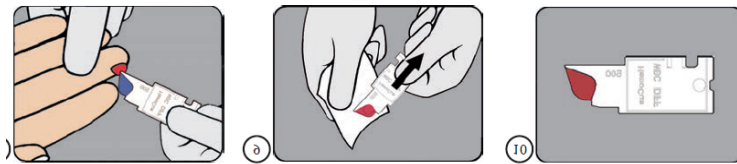


Figure 1: Filling cuvette and wiping excess blood¹.

14. Apply cotton ball to puncture site. Ask consumer (if able) to hold firmly until bleeding stops
15. Place cuvette into the cuvette holder immediately and gently tap handle to right. The holder will close and immediately start analysis
16. Refer to manufacturer's video available at <https://www.youtube.com/watch?v=PFZPMgCl6KY>

HemoCue® Capillary sampling cont'd

Recording the results

1. The results will show in the display window of the machine and print automatically when the test is complete
2. The HemoCue® system will display a flag or error code if a problem is detected within the system or sample
3. Remove cuvette from analyser and dispose in identified yellow clinical waste bins as per **RFBG Sharps Management Policy**
4. On completion of the procedure remove gloves and perform hand hygiene. Refer to **RFBG Hand Hygiene Policy**
5. Don clean gloves and wipe over HemoCue® Machine and the cuvette holder after each day of use
 - Turn off the analyzer
 - Place the cuvette moving arm in loading position
 - Remove cuvette holder by lifting it straight up
 - Clean the cuvette holder with isopropyl 70% alcohol wipes or detergent wipe and place on a clean bench
 - Note: do not autoclave
 - If the optical parts are stained an error code is displayed. Clean according to Maintenance/ Optical Parts section (refer to p. 27 [HemoCue® WBC DIFF Operating Manual](#))
 - Wait 15 minutes before replacing the cuvette holder
 - Place the cuvette moving arm in loading position before replacing the cuvette holder
 - Clean the cover with isopropyl 70% alcohol wipes/detergent wipe.
6. Document the HemoCue® reading in the Clopine Central™ database
7. Consumer's name, date, WBC count, NCC, HR, BP, Wt, Clozapine dose
8. Attach printed results to the corresponding consumer's Clozapine blood count record form and arrange review by treating MO
9. Record results on the consumer's *Clozapine Monitoring Form Part A* and document in the healthcare record
10. Any results returned that are in the Amber or Red range must be immediately referred for venous sampling to confirm accuracy and ensure escalation as per [Blood Testing](#) section above.

Storage conditions: HemoCue® WBC DIFF

- The HemoCue® WBC DIFF microcuvettes must be used prior to their expiry date that is printed on each package
- The microcuvettes are stored at room temperature (15 - 35 °C) and are sensitive to temperate and humidity. Do not refrigerate
- Once the microcuvette is unwrapped it must be used within 10 minutes.

HemoCue® Capillary sampling cont'd

Quality control: Analyser self test

Each time the HemoCue® WBC DIFF machine is turned on, it automatically verifies the performance of the optronic unit. If the test fails, the instrument will display an error code (Err61). Refer to the [HemoCue® WBC DIFF Operating Manual](#) for trouble shooting and an explanation of the error codes.

Refer to [HemoCue® WBC DIFF Operating Manual](#) for how to conduct the quality control test.

- Store quality control material upright refrigerated at 2 - 8°C
- Protect vials from overheating and freezing
- Unopened vials are stable until the expiration date. Record date vial opened near expiry date
- Opened vials are stable for 30 days provided they are handled properly
- The low (level 1), normal (level 2) and high (level 3) quality controls must be tested monthly
- Quality control must be tested for each new low number of microcuvettes (refer to the [HemoCue® WBC DIFF Operating Manual](#), p. 45)
- If the quality control test fails it must be repeated to obtain a pass result before consumer samples are tested.

Maintenance: HemoCue® WBC DIFF

The analyser is factory calibrated and does not require further calibration. For details on performing the maintenance refer to the [HemoCue® WBC DIFF Operating Manual](#) maintenance section.

Compliance monitoring

Compliance will be monitored annually (as a minimum) by the MHD Co-Directors and tabled at relevant Divisional specific Clinical Quality and Safety meetings via:

- A review of Datix Clinical Incident Management System (CIMS) and complaints systems
- Audits relating to clozapine management as per RPBG Performance Monitoring Schedule

Facilitator

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6. *Clopine® Product Information*, approved by the Therapeutics Goods Administration 14 July 2004, amended 12 June 2009. [CLOPINE PRODUCT INFORMATION](#)
7. Clopine®.Patient Monitoring Service - <https://www.clopine.com.au/ClopineCentral/>

Related policies, practice standards, clinical guidelines

RPBG Policy Hub

- [Prescribing Policy](#)
- [Medication Management and Storage Clinical Guideline](#)
- [Medication Preparation and Routes of Administration CPS](#)
- [High Risk Medication Policy](#)
- [Recognising and Responding to Acute Deterioration in Mental State, Cognition and Behaviour SOP](#)
- [Recognising and Responding to Acute Deterioration Policy](#)
- [CMHS Provision Responding to Respiratory Virus Illness SOP](#)
- [Sharps Management Policy](#)
- [Hand Hygiene Policy](#)
- [RVI and Infection Prevention and Management Clinical Guideline](#)

Department of Health Policy Hub

- [WA High Risk Medication Policy](#)
- [Mental Health Charts and Clozapine Resources](#)
- [Medication Review Policy](#)
- [WA Health Consent to Treatment Policy](#)
- [Statewide Standardised Clinical Documentation \(SSCD\)](#)

Other

- [Mental Health Act 2014](#)
- [Charter of Mental Health Care Principles \(MHA 2014\)](#)
- [Chief Psychiatrists Standards for Clinical Care 2015](#)
- [National Medicines Policy 2022](#)
- [Poisons Standard June 2018](#)
- [Clinical Guidelines for the Physical Care of Mental Health Consumers 2010](#)

Related forms

- WA Adult Clozapine Initiation and Titration Chart
- Clozapine Blood Count Record Form
- Clozapine Monitoring Forms A, B, C
- Consent to Clozapine Treatment Form
- Adult Observation and Response Chart (AORC)

Related National Standards

ACSQHC NSQHS Standards 2nd Edition (2021): Standard 4: Medication Safety; Standard 5: Comprehensive Care; Standard 6: Communicating for Safety; Standard 7: Blood Management

NSMHS Standards (2010): Standard 2: Safety;

Appendix I: Bidi Wungen Kaat (Transitional Care Unit) Clozapine Monitoring Guidelines

Pre-Clozapine Workup	- As per Inpatient Commencement of Clozapine section above - Ensure that all consumers complete the Consent to Clozapine Treatment Form prior to commencement - Add Clinical Alert on webPAS and PSOLIS/WebPSOLIS alert for clozapine
All monitoring recommendations are required as a minimum - increase as clinically indicated. Use the WA Clozapine Initiation and Titration Chart to guide additional physical health monitoring for weeks 1 – 4. Continue haematological monitoring weekly (FBC) for the full 18 weeks.	
Day 1	- Monitor vital signs (including lying and standing BP) prior to the first dose of clozapine - Then monitor vital signs (including lying and standing BP) hourly for 6 hours post initiation, as per the Clozapine initiation and titration chart. Then every 6 hours for the first 24hrs - Encourage consumer to stay on unit for the first 24hrs post-initiation - Ensure vital signs are recorded on <i>Clozapine initiation and titration chart and TCU AORC</i>
Days 2-7 (Week 1)	- Monitor vital signs (including lying and standing BP) twice daily and review of clozapine related side-effects - Ensure vital signs are recorded on <i>Clozapine initiation and titration chart and TCU AORC</i> - All consumers should be encouraged to stay on the unit over the weekend for continued monitoring No dose increases prescribed over weekend and Public Holidays
Week 2	- Monitor vital signs (including lying and standing BP) daily and review of clozapine related side-effects - Ensure vital signs are recorded on TCU AORC No dose increases prescribed over weekend and Public Holidays
Week 3	- Monitor vital signs (including lying and standing BP) daily and review of clozapine related side-effects. - Ensure vital signs are recorded on TCU AORC No dose increases prescribed over weekend and Public Holidays

Week 4	• Monitor vital signs (including lying and standing BP) daily and review of clozapine related side-effects • Ensure vital signs are recorded on TCU AORC • No dose increases prescribed over weekend and Public Holidays
Weeks 5-18	• Ensure twice weekly monitoring of vital signs (including lying and standing BP) and review of clozapine related side-effects. Increase monitoring if clinically indicated • Completion of <i>Clozapine Monitoring Form Part A</i> weekly, and ensure vital signs are also recorded on TCU AORC
Ongoing (after 18 weeks)	• Weekly monitoring of vital signs (including lying and standing BP) and review of clozapine related side-effects • Complete <i>Clozapine Monitoring Forms A & B</i> monthly post-blood test to guide physical health monitoring protocols • Ensure vital signs are also recorded on TCU AORC